NUR 774 Exam 3: Memory Hooks

Mnemonics, plain-English hooks, and test strategies. Exam: November 17, 2026 (40 questions, 5 on immunizations).

RESPIRATORY

1. Strep throat (GAS)

- **STREP** = **S**udden sore throat, **T**emp (fever), **R**ed tonsils ± exudate/petechiae, **E**nlarged ANTERIOR nodes, **P**ain swallowing (kids may add tummy ache/headache)
- **"Looks like a COLD → not strep": C**ough, **O**ral ulcers, **L**aryngitis (hoarse), **D**rippy nose, plus pink eye
- **"Negative rapid at 3+? Culture it."**
- **ASO = "Already Strep Occurred"**: shows PAST infection, not the test for today's sore throat.
- Amoxicillin or penicillin for **10 days**. Bicillin IM: **27 kg splits it** (under 27 kg = 600,000 units; 27 kg or more = 1.2 million).
- Back to school after **24 hrs of antibiotics + no fever**.

2. Strep complications

- **"Penicillin protects the Heart, not the Kidney."** Antibiotics prevent rheumatic fever but NOT reliably PSGN.
- **PSGN = "Cola, Puffy, Pressure"**: tea-colored urine, edema, high BP.
- **Peritonsillar abscess:** hot-potato voice, trismus, **"the uvula runs AWAY from the abscess."**
- **Retropharyngeal abscess:** younger child, stiff neck/torticollis, drooling, airway risk.
- **Mono: "Front nodes = Strep, Back nodes = Mono."** Fatigue + big spleen → **no contact sports**.
- **Airway trumps everything:** drooling, stridor, toxic look → stop the throat exam, get urgent help.

3. Pneumonia: organism by age

- **Newborn → "Born through the canal":** GBS + E. coli.
- **1–3 months → Chlamydia's 3 C's: C**hoppy (staccato) cough, **C**ool (no fever), **C**onjunctivitis first.
- **Preschool** → viruses (RSV, flu) + S. pneumoniae.
- **School age → "My School = Mycoplasma"** (walking pneumonia): gradual, headache, dry cough → **M**acrolide (azithro).
- **After the flu → Staph** (incl. MRSA).
- **"Typical → Amox (90 mg/kg/day BID); Atypical → Azithro."** Reassess at **48–72 hrs**.
- Lower-lobe pneumonia can look like **belly pain**. Viral = supportive care, no antibiotics.

4. Asthma

- **Intermittent = "Rule of 2s":** ≤2 days/wk, ≤2 nights/month, SABA ≤2 days/wk, FEV1 >80%.
- **Severity ladder:** Intermittent (≤2) → Mild (>2, not daily) → **Moderate = DAILY** → Severe = ALL DAY.
- **FEV1: 80 / 80 / 60–80 / under 60.**
- **"Worst feature wins"**: classify by the single most severe feature.
- Ages 0–4 nights: none / 1–2 per month / 3–4 per month / more than 1 per week.

- **Meds: Albuterol = Ambulance** (rescue) · **ICS = Insurance** (daily; rinse & spit) · **LABA never Alone** · **Montelukast = Mood warning** · Prednisone = for flares, not a bronchodilator.
- **Peak flow = traffic light:** Green 80%+, Yellow 50–79%, Red under 50%.
- **"A silent chest is a scream"**: less wheeze + tired child = respiratory failure.
- **Dry-powder inhaler = no spacer.** MDI + spacer works even in toddlers (with mask).

5. Croup

- **3 S's: Seal bark, Stridor (inspiratory), Steeple sign**. Age 6 mo–3 yrs. Parainfluenza ("**Para-croup**"). Worse at night.
- **"Dex for all, Epi for stridor at rest, watch 2–4 hrs before the door."** Dexamethasone 0.6 mg/kg.
- Albuterol and antibiotics do NOT help croup. No x-ray needed.
- **Epiglottitis = 4 D's: Drooling, Dysphagia, Dysphonia (muffled), Distress** + tripod → **don't look in the throat.**

6. Pertussis and foreign body

- **Pertussis stages: "Cold → Cough → Calm"** (Catarrhal → Paroxysmal → Convalescent).
- **Infants: no whoop**, they get apnea and turn blue.
- Azithromycin. **School: "5 days on drugs, 21 days without."**
- **Foreign body = "Sudden + one Side"** (unilateral wheeze or decreased breath sounds).
- **"Normal x-ray doesn't clear the airway"** (food doesn't show). Bronchoscopy. **No blind finger sweeps.**

7. Other respiratory

- **Bronchiolitis (RSV, under 2) = 3 S's: Suction, Sips, Saturation (O2).** NO albuterol, steroids, antibiotics.
- **"Flu hits like a truck"**: abrupt fever, aches, chills. A cold is gradual and runny.
- **Bacterial sinusitis: "10 days, Double-dip, 39 for 3"** (>10 days no improvement; worse after improving; ≥39°C with pus for 3 days).
- Allergic rhinitis: pale boggy turbinates, allergic shiners, itchy.
- **No honey under age 1.** No OTC cough/cold mixes in young kids.

EAR

8. Acute otitis media (AOM)

- **Bugs = "Stop Hurting Me": S**. pneumo, **H**. influenzae, **M**oraxella.
- **"Bulge = AOM. Red alone = Rubbish."** Crying makes the eardrum red too.
- **Who gets antibiotics:** Under 6 months → always. Under 2 + **both ears** → always. Under 2 + one ear → treat OR watch. 2 and older → watch OR treat.
- **Severe = "39° or 48 hrs"** (or ear drainage) → treat.
- **Amox 80–90 mg/kg/day BID.**
- **Augmentin = 3 C's:** amox in last **30** days, **C**onjunctivitis (purulent) + AOM, **C**an't clear (recurrent, failed amox).
- **Duration "10–7–5":** under 2 or severe = 10 days, age 2–5 = 7, age 6+ = 5–7.
- **Always treat the pain.** Observation needs follow-up in 48–72 hrs.

9. Other ear and hearing

- **OME = "Fluid without Fire"**: watch ~3 months, no antibiotics.
- **Otitis externa (swimmer's ear) = "Pull the ear, it hurts"** (tragus/pinna). Ear drops, keep dry.
- **Mastoiditis = "ear sticks out + red behind it"** → urgent.
- **Conductive = Cerumen & fluid** (outer/middle ear). **Sensorineural = Snail (cochlea) & nerve.**
- **Hearing 1–3–6:** screen by 1 month, diagnose by 3, intervene by 6.
- **Never irrigate the 3 B's: Battery, Beans (vegetable matter), Broken eardrum.**
- **Recurrent AOM = 3 in 6 months or 4 in 12 months.** No routine prophylactic antibiotics.

EYE

10. Common eye disorders

- **Bacterial = "Bacon grease"** (goopy, glued lids) · **Viral = Watery + cold** · **Allergic = Itchy** (not contagious).
- **Pink eye + ear infection = H. flu → Augmentin.**
- **"A Stye Stings, a Chalazion is Chill"** (painful/acute vs painless/persistent). Warm compresses, **don't squeeze**.
- **Orbital cellulitis = 4 P's: Pain with eye movement, Proptosis, Poor vision, Paralysis (restricted movement)** → ER, IV antibiotics. Preseptal = lid only, eye moves fine.
- Contact lens + painful red eye → urgent (keratitis). No steroid drops on an undiagnosed red eye.

11. Vision screening and serious eye problems

- **"MY-opia: I only see near MY face"** (nearsighted). Hyperopia = farsighted.
- **Amblyopia = "the brain Ambles away from the weak eye."** Strabismus = misalignment (can cause amblyopia).
- **Passing lines: age 3 = 20/50, age 4 = 20/40, age 5+ = 20/32.** "Child grows, number shrinks." Test each eye separately.
- **White reflex = retinoblastoma until proven otherwise.**
- **Newborn eye infection: "Gonorrhea Goes fast"** (days 2–5, ceftriaxone) · **"Chlamydia Comes later"** (days 5–12, ORAL meds, watch for pneumonia).
- **Chemical burn = irrigate FIRST.** Globe rupture = shield, no pressure.

CARDIAC

12. Fetal circulation and CHD basics

- **3 shunts:** Ductus **V**enosus skips the li**V**er · Foramen **O**vale = RA → LA ("**O**pen door between atria") · Ductus **A**rteriosus = PA → **A**orta (skips the lungs).
- **Acyanotic L→R = the "D's": ASD, VSD, PDA, AVSD.** Obstructive (acyanotic): coarctation, AS, PS.
- **Cyanotic = the 5 T's: T**runcus (1 trunk), **T**ransposition (2 vessels switched), **T**ricuspid atresia (3), **T**etralogy (4 parts), **T**APVR (5 letters).
- **"Prostaglandin keeps it Patent; Indomethacin Interrupts (closes) it."**
- **"Feeding is the infant's treadmill"**: sweating with feeds = heart failure.

13. ASD and VSD

- **ASD → "Always Split, Doesn't change"** = wide **fixed split S2**. Often found later.
- **VSD → "Very loud"**: harsh holosystolic murmur, lower left sternal border. **"Small hole, loud howl"** (loudness ≠ severity).
- Large VSD gets symptomatic **weeks after birth** as lung resistance falls.
- **AV canal defect ↔ Down syndrome.** Echo confirms.

14. PDA and obstructive lesions

- **PDA = "Machinery"**: continuous murmur, bounding pulses, wide pulse pressure.
- **Coarctation = "Strong upstairs, weak downstairs"**: high arm BP, weak/delayed femoral pulses. Newborn → shock as ductus closes.
- **Aortic stenosis:** exertional chest pain/fainting → **no automatic sports clearance.**
- **Pulmonary stenosis:** murmur at upper left sternal border ± click.

15. Cyanotic heart disease

- **Tetralogy = "PROV": Pulmonary stenosis, RVH, Overriding aorta, VSD.** Boot-shaped heart.
- **Tet spell → knee-chest ("tuck like a frog")**, calm, O2, emergency. Older kids squat.
- **TGA = "Two separate circles"**: needs mixing (PGE1, balloon septostomy) → arterial switch. **Never close that PDA.**
- **HLHS = duct-dependent**: shock as PDA closes.
- **PGE1 → watch for apnea.** Central cyanosis = tongue/lips.

16. Murmurs

- **Innocent = 7 S's:** Soft, Systolic, Short, Single, Small area, Sweet, Sensitive (changes with position).
- **Red flags: "Diastolic is always Dangerous"** + harsh, holosystolic, grade 3+, thrill, symptoms, family history of sudden death.
- **Levine: "4 = you can FEEL it (thrill)."** 1 barely · 2 soft · 3 loud no thrill · 4 thrill · 5 stethoscope edge · 6 stethoscope off chest.
- **Still's** = musical, louder lying down. **Venous hum** = continuous, disappears lying down.
- **HCM gets LOUDER standing/Valsalva** (most innocent murmurs get softer).

17. Cardiac diagnostic tests

- **"Echo = Eyes (anatomy/holes); ECG = Electricity (rhythm)."** An ECG can't show a hole.
- **"Holter = Hold on for a day"** (24–48 hrs, frequent symptoms). **"Event = Every now and then"** (rare symptoms).
- **Labs:** ASO = past strep · ESR/CRP = inflammation · BNP = heart failure strain · Troponin = heart muscle injury · Blood cultures **before** antibiotics for endocarditis.
- **After cardiac cath:** check site for bleeding + **pulses below it**.
- Normal ECG ≠ no CHD. Normal pulse ox ≠ no acyanotic shunt.

18. Kawasaki: "CRASH and BURN"

- **C**onjunctivitis (both eyes, no pus) · **R**ash · **A**denopathy (one side, ≥1.5 cm) · **S**trawberry tongue / cracked lips · **H**ands & feet (swollen, later peeling) · **BURN** = fever 5+ days.
- Need **fever + 4 of 5.** Usually under 5 years.
- **"Kawasaki Crowns the Coronaries"** → echo (serial).

- **IVIG 2 g/kg within 10 days + aspirin** (Kawasaki is the aspirin exception).
- **"2 grams → wait 11 months"** for MMR/varicella.

19. Acute rheumatic fever: "JONES"

- **J**oints (migratory) · **O** (♥ shape = carditis) · **N**odules (subcutaneous) · **E**rythema marginatum · **S**ydenham chorea.
- **Minor = "FEAR": F**ever · **E**levated ESR/CRP · **A**rthralgia · **R**hythm (prolonged PR).
- **2 major OR 1 major + 2 minor** + proof of prior strep. **Don't double-count** arthritis+arthralgia or carditis+PR.
- **"Kawasaki = Coronaries, under 5. Rheumatic = Valves (mitral), school age, weeks after strep."**
- Prevention: monthly benzathine penicillin (secondary prophylaxis). Not the same as dental prophylaxis.

20. Other cardiac

- **Pericarditis = "praying position"**: better leaning forward, worse lying flat.
- **Myocarditis = heart rate too fast for the fever.**
- **Endocarditis:** fever + heart risk + new murmur → blood cultures + echo.
- **"Faints while running = Referral."** Vasovagal has warning signs + a trigger.
- **Marfan:** tall, long fingers, dislocated lens → **aortic root** (dissection).
- **Dental prophylaxis NOT for plain ASD/VSD/MVP.**
- **BP:** under 13 = percentiles; 13+ = adult cutoffs.

MEDICATIONS

21. Doses and math

- **Tylenol: 10–15 mg/kg every 4–6 hrs** (max 5 doses/day).
- **Ibuprofen = "the 6s": 6 months and older, every 6–8 hrs**, 5–10 mg/kg.
- **No aspirin (Reye's)**, except Kawasaki.
- **"Per DAY → Divide. Per DOSE → Don't."**
- **Liquid formula: mg needed × 5 ÷ (mg per 5 mL) = mL.**
- **lb ÷ 2.2 = kg.**
- Example: 20 kg, Tylenol 15 mg/kg = 300 mg; 300 × 5 ÷ 160 = **9.4 mL**.
- Example: 20 kg, AOM amox 90 mg/kg/day = 1,800 mg/day ÷ 2 = **900 mg per dose**.

IMMUNIZATIONS (5 questions = easy points)

22. Routine schedule

- **2, 4, 6 months: "Dad Is Having Pizza Rolls"** = **D**TaP, **I**PV, **H**ib, **P**CV, **R**otavirus. Flu starts at 6 months.
- **12–15 months = "Live birthday party":** MMR + Varicella + Hib/PCV boosters + HepA.
- **4–6 years (school entry) = "DIMV":** DTaP, IPV, MMR, Varicella.
- **11–12 years = "Teens Meet HPV":** Tdap, MenACWY, HPV (HPV can start at 9). MenACWY booster at 16.

- **Rotavirus: start by 15 weeks (14 wk 6 d), finish by 8 months.**
- **HepA:** 2 doses, 6 months apart.

23. Catch-up and special situations

- **"Live shots: same day or wait a month (28 days)."** Non-live can be given any time.
- **Catch-up: NEVER restart a series.**
- **HepB "4–8–16":** 4 wks (dose 1→2), 8 wks (2→3), 16 wks (1→3); last dose at 24+ weeks old.
- **HPV:** started under 15 = 2 doses (6–12 months apart); 15+ = 3 doses.
- **Steroid "2–20–14":** ≥2 mg/kg/day or ≥20 mg/day for ≥14 days → wait 1 month for live vaccines. Inhaled steroids = no delay.
- **Mild cold / on antibiotics = still vaccinate.** Moderate/severe illness = wait. Anaphylaxis = no.
- Immunocompromised family member at home → still give MMR + varicella.

SCHOOL AND FAMILY TEACHING

24. Return to school

- **Strep:** 24 hrs antibiotics + no fever.
- **Pertussis:** 5 days antibiotics (21 days if untreated).
- **Chickenpox:** all lesions crusted.
- **Fifth disease and ear infections:** no exclusion.
- **Nosebleed:** lean **FORWARD**, pinch the soft part **10 minutes**. Never tilt back.

TEST-TAKING STRATEGIES

How to attack the questions

- **Airway first, always.** Drooling, silent chest, tripod, apnea → the answer protecting breathing wins. Stabilize before x-rays.
- **Watch for the word "alone."** Red eardrum alone, ASO alone, pus alone, loud murmur alone: never enough.
- **Underline the age.** It decides the pneumonia bug, the AOM rule, the asthma night thresholds, and the vaccine.
- **Assess before you act.** If the stem lacks data, the answer is usually "assess."
- **Say the question in your own words** before reading choices, then cross off 2.
- **Don't change answers** unless you find a fact you misread.
- **Do the math twice.** Write mg → mL on scratch paper; ask "is this a reasonable volume?"

Trap list (the exam's favorite tricks)

- PGE1 OPENS the PDA (it doesn't close it).
- Albuterol does nothing for croup or bronchiolitis.
- Antibiotics don't prevent PSGN.
- Normal x-ray doesn't rule out a foreign body.
- mg/kg/DAY must be divided.
- A delayed vaccine series is never restarted.

- Fewer wheezes can mean WORSE asthma.
- Peeling in Kawasaki comes LATE; don't wait for it.

7-day study plan

- **Days 1–2:** Respiratory + ear.
- **Days 3–4:** Cardiac.
- **Day 5:** Eye, meds, immunizations.
- **Day 6:** Practice questions + dose math.
- **Day 7:** Write every mnemonic from memory on a blank page; review only what you missed. Sleep.
- **Every night:** say the mnemonics out loud for 10 minutes (to someone or the mirror). Teaching it back locks it in.